Assessment of Frailty and Health Outcomes in Patients with COPD; a retrospective study using the Electronic Frailty Index

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Introduction

Why Frailty Matters?

- Acute exacerbations associated with decline in lung function, worse quality of life, increased mortality
- Aging population: $\sim 16\%$ of adults ≥ 65 are frail
- Potential to predict adverse outcomes independent of age and comorbidity
- **Electronic Frailty Index (eFI)** derived from EHR data

Study Goal: Replicate & extend Pajewski et al. (2019) by assessing the association between frailty (stratified by eFI) and health outcomes/utilization in patients with COPD from their EHR/claims data.

Research Objectives

1. **Describe baseline characteristics** by frailty category
2. **Test predictive ability** of eFI and 36 clinical components
3. **Compare performance** to prior literature

Research Question

Can an eFI computed from routinely collected EHR variables predict hospitalization and mortality?

Electronic Frailty Index: Concept

Deficit Accumulation Theory

- Frailty reflects cumulative health deficits across multiple systems.
- Each deficit adds to overall vulnerability.
- Derived from pre-existing records (ICD codes, labs, meds).
- Predicts hospitalization, mortality, care needs

eFI Calculation

- Quantifies health deficits from EHR data
- 36 clinical deficits counted.
- eFI $= \frac{\text{deficits present}}{36 \text{ total}}$
- Range: 0 (fit) to 1 (most frail).

eFI Categories

Metric	Fit	Pre-Frail	Frail
eFI cutoff	≤ 0.10	0.10–0.21	> 0.21
Our sample	19.4%	47.9%	32.6%

Study Population

Cohort Flow

Retrospective cohort study using EHR data at Atrium Health Wake Forest Baptist

Inclusion Criteria:

- Age $\geq$55 as of 1/1/2018
- With COPD $\geq$2 billing codes in prior 2 years (J41, J42, J43, J44)
  - Initial $N = 24,929$
  - Missing eFI: $6,651$ (26.7%)
  - **Analytic sample = 18,278**

Timeframe:

- **eFI calculation:** January 1, 2016 – December 31, 2017
- **Outcome follow-up:** January 1, 2018 – December 31, 2019

Results

The eFI Had Modest Predictive Ability in COPD Patients

Hospitalization AUC = 0.582-0.586 (barely better than random chance at 0.50)

Mortality AUC = 0.754-0.654 but limited events (unstable estimate)

EFI Variables and Adverse Outcomes

Complete Case Analysis (N = 18,278)

EFI Component	Hospitalized < 6 Months			Hospitalized < 12 Months			Mortality < 6 Months			Mortality < 12 Months		
	AUC	OR (95% CI)	p	AUC	OR (95% CI)	p	AUC	OR (95% CI)	p	AUC	OR (95% CI)	p
Overall EFI Score	0.582	**43.47 (33.03–91.89)**	<0.001	0.586	**48.68 (39.29–99.08)**	<0.001	0.754	**3.01 (0.00–664.77)**	0.402	0.654	**0.01 (0–11.9)**	0.206
Heart Failure	0.632	**2.92 (2.63–3.23)**	<0.001	0.631	**2.83 (2.57–3.12)**	<0.001	0.666	7.95 (0.67–89.91)	0.091	**0.657**	**3.91 (0.26–19.63)**	**0.016**
Diabetes	0.569	**2.08 (1.88–2.3)**	<0.001	0.581	**2.12 (1.92–2.33)**	<0.001	0.513	0.96 (0.07–8.66)	0.994	0.572	0.54 (0.14–1.64)	0.296
Dyspnea	0.571	**1.81 (1.64–2.01)**	<0.001	0.571	**1.84 (1.66–2.03)**	<0.001	0.610	0.3 (0.03–1.5)	0.150	0.524	0.64 (0.29–2.03)	0.735
Delirium	0.562	**2.31 (2.05–2.6)**	<0.001	0.559	**2.25 (2–2.53)**	<0.001	0.568	0.48 (0–4.1)	0.583	0.539	1.92 (0.49–5.79)	0.316
Atrial Fibrillation	0.566	**2.07 (1.84–2.32)**	<0.001	0.559	**2.05 (1.84–2.3)**	<0.001	0.562	0.39 (0–3.3)	0.462	0.511	1.02 (0.2–3.39)	0.993
Renal Disease	0.556	**1.68 (1.51–1.86)**	<0.001	0.553	**1.64 (1.48–1.82)**	<0.001	0.622	0.2 (0–1.71)	0.175	0.528	1.61 (0.27–5.46)	0.726
Anemia	0.542	**1.65 (1.47–1.85)**	<0.001	0.542	**1.64 (1.47–1.84)**	<0.001	0.574	2.43 (0.42–16.94)	0.287	0.514	1.33 (0.34–4.53)	0.648
Syncope	0.520	**1.94 (1.66–2.26)**	<0.001	0.527	**1.87 (1.6–2.17)**	<0.001	0.530	0.94 (0.01–7.92)	0.963	0.533	2.43 (0.49–6.13)	0.246
Falls	0.538	**1.62 (1.44–1.83)**	<0.001	0.537	**1.6 (1.42–1.79)**	<0.001	0.501	1.35 (0.14–6.75)	0.753	0.548	0.56 (0.06–2.24)	0.451
Fracture	0.532	**1.42 (1.27–1.59)**	<0.001	0.529	**1.38 (1.23–1.54)**	<0.001	0.522	1.02 (0.11–5.13)	0.979	0.589	0.42 (0.05–1.7)	0.255
Hypertension	0.542	**1.77 (1.54–2.04)**	<0.001	0.543	**1.78 (1.56–2.05)**	<0.001	0.662	0.28 (0.04–1.3)	0.099	0.606	0.36 (0.13–1.05)	0.061
Hypothyroidism	0.546	**1.66 (1.48–1.85)**	<0.001	0.547	**1.66 (1.49–1.85)**	<0.001	0.604	0.28 (0–2.47)	0.216	0.503	1.15 (0.29–3.49)	0.816
Urinary Issues	0.539	**1.39 (1.25–1.54)**	<0.001	0.539	**1.38 (1.25–1.53)**	<0.001	0.650	5.21 (0.02–137)	0.061	0.530	0.78 (0.28–2.21)	0.638
CAD	0.530	**1.38 (1.24–1.52)**	<0.001	0.540	**1.38 (1.26–1.53)**	<0.001	0.630	0.38 (0.04–1.9)	0.256	0.595	0.6 (0.18–1.71)	0.346
Depression	0.522	**1.27 (1.09–1.34)**	<0.001	0.522	**1.27 (1.09–1.33)**	<0.001	0.504	9.53 (0.05–2.63)	0.459	0.564	4.58 (0.15–1.77)	0.393
Chronic Pain	0.519	**1.24 (1.1–1.4)**	<0.001	0.521	**1.28 (1.14–1.45)**	<0.001	0.548	0.98 (0.12–3.23)	0.480	0.595	0.41 (0.15–1.18)	0.097
Rheumatoid Arthritis	0.520	**1.25 (1.13–1.39)**	<0.001	0.520	**1.25 (1.13–1.38)**	<0.001	0.619	0.16 (0–1.22)	0.083	**0.618**	**0.21 (0.02–0.85)**	**0.026**
Dizziness	0.519	**1.24 (1.1–1.39)**	<0.001	0.521	**1.26 (1.13–1.41)**	<0.001	0.615	0.28 (0–2.23)	0.270	0.577	0.38 (0.04–1.64)	0.202
Valvular Disease	0.519	**1.41 (1.22–1.62)**	<0.001	0.517	**1.38 (1.19–1.58)**	<0.001	0.555	0.62 (0–5.23)	0.725	0.555	0.28 (0–2.07)	0.269
Cancer (Other)	0.515	**0.86 (0.76–0.96)**	0.009	0.516	**0.85 (0.76–0.95)**	0.004	0.547	0.77 (0.08–3.66)	0.772	0.559	0.56 (0.11–1.88)	0.361
Osteoporosis	0.516	**0.72 (0.61–0.84)**	<0.001	0.519	**0.71 (0.6–0.82)**	<0.001	0.566	0.49 (0–4.13)	0.589	0.566	0.22 (0–1.64)	0.174
Weight Loss	0.516	**0.82 (0.72–0.93)**	0.002	0.515	**0.78 (0.69–0.88)**	<0.001	0.602	0.3 (0–2.56)	0.332	0.535	0.79 (0.15–2.62)	0.721
Stroke	0.510	**1.18 (1.03–1.36)**	0.021	0.510	**1.19 (1.03–1.36)**	0.015	0.567	0.5 (0–4.19)	0.596	0.567	0.22 (0–1.66)	0.179
Peptic Ulcer	0.506	**1.44 (1.1–1.86)**	0.006	0.506	**1.45 (1.12–1.86)**	0.006	0.515	3.56 (0.02–21.67)	0.574	0.521	3.7 (0.4–15.15)	0.199
Peripheral Vascular Disease	0.513	**1.13 (1.01–1.25)**	0.028	0.512	**1.12 (1.01–1.25)**	0.027	0.572	0.61 (0.06–3.04)	0.572	0.546	0.68 (0.17–2.01)	0.511
Incontinence	0.507	**1.3 (1.06–1.58)**	0.014	0.508	**1.31 (1.07–1.58)**	0.009	0.528	1.91 (0.01–11.05)	0.861	0.508	1.89 (0.21–7.72)	0.497
Acute MI	0.523	**1.68 (1.43–1.96)**	<0.001	0.521	**1.64 (1.4–1.9)**	<0.001	0.541	0.67 (0.01–7.37)	0.923	0.541	0.39 (0–2.91)	0.445
Thyroid Disease	0.502	0.98 (0.87–1.1)	0.725	0.506	0.94 (0.83–1.05)	0.261	0.631	0.24 (0–2.02)	0.230	0.592	0.62 (0.12–2.07)	0.488
Vision Loss	0.505	1.11 (0.94–1.31)	0.203	0.504	1.1 (0.94–1.29)	0.242	0.548	0.71 (0.01–6.01)	0.807	0.549	0.32 (0–2.38)	0.337
Hearing Loss	0.504	0.96 (0.85–1.08)	0.501	0.502	0.97 (0.86–1.09)	0.587	0.613	0.27 (0–2.24)	0.273	0.545	1.69 (0.12–3.3)	0.577
Dementia	0.506	1.11 (0.95–1.29)	0.171	0.507	1.13 (0.98–1.3)	0.093	0.582	0.52 (0–4.38)	0.422	0.507	1.35 (0.26–4.5)	0.673
Melanoma	0.501	0.85 (0.45–1.45)	0.562	0.501	0.79 (0.42–1.34)	0.391	0.504	8.85 (0.07–75.34)	0.261	0.504	3.96 (0.03–29.82)	0.431
Blood Cancer	0.500	0.97 (0.87–1.08)	0.850	0.501	0.92 (0.84–1.29)	0.649	0.511	3.57 (0.03–30.27)	0.465	0.511	1.6 (0.11–11.97)	0.761
Parkinson's Disease	0.500	1.07 (0.7–1.56)	0.740	0.501	1.11 (0.75–1.6)	0.584	0.508	4.93 (0.04–41.89)	0.379	0.508	2.21 (0.02–16.57)	0.624
Liver Disease	0.514	**1.57 (1.21–2.02)**	<0.001	0.513	**1.54 (1.2–1.97)**	<0.001	0.555	6.57 (0–19.74)	0.825	0.555	0.71 (0–4.12)	0.311

Bold text indicates p < 0.05 (statistically significant).
AUC cells color-coded: White = poor discrimination (< 0.56), Gold = good discrimination (≥ 0.65+).
EFI Score modeled with RCS (5 knots). Other components modeled with unadjusted Firth's penalized logistic regression.
AUC = Area Under ROC Curve; OR = Odds Ratio; CI = Confidence Interval.

Figure 1. **Discrimination Summary** — Hospitalization AUC [VALUE] indicates **modest predictive performance**; Mortality AUC [VALUE] but limited power due to sparse events.

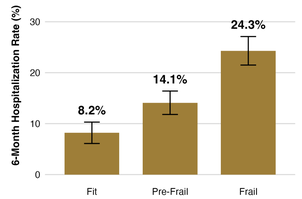


Figure 2. **Healthcare Utilization by Frailty** — Despite modest discrimination, stratification works: [XX]% → [XX]% → [XX]%. Frail vs Fit OR = [VALUE] (95% CI: [LOWER]–[UPPER]).

RCS Predictions with Firth Penalization: Continuous eFI Score and Adverse Outcomes

		Hospitalization				Mortality			
eFI Percentile	eFI Score	6-mo Prob	OR vs 10th	12-mo Prob	OR vs 10th	6-mo Prob	OR vs 10th	12-mo Prob	OR vs 10th
10th	0.070	6.049608%	1.00	6.377143%	1.00	0.3025197%	1.00	0.3195852%	1.00
25th	0.112	7.842594%	1.32	8.358020%	1.34	0.2816990%	0.93	0.2723033%	0.85
50th	0.163	8.891486%	1.52	9.468676%	1.54	0.2387128%	0.79	0.2543891%	0.80
75th	0.218	10.506938%	1.82	11.437263%	1.90	0.2657802%	0.88	0.2093875%	0.65
90th	0.269	12.440430%	2.21	13.511561%	2.29	0.2776121%	0.92	0.2298960%	0.72
95th	0.302	13.813836%	2.49	14.854114%	2.56	0.2744705%	0.91	0.2690442%	0.84

Modeled with restricted cubic splines (5 knots) using Firth's penalized logistic regression.
Probabilities are predicted risks; ORs are relative to the 10th percentile eFI score.

Figure 3. **RCS Predicted Probabilities** — Risk accelerates above 75th percentile; 95th vs 10th percentile ≈ [XX]× odds. However, overall model AUC remains modest.

Outcomes Measured

Primary Outcome:

- **COPD Exacerbations @ 12 months** — Hospital admissions or ER visits with ICD-10 J41–J44

Secondary Outcomes:

- All-cause Mortality (binary)
- Pulmonary Mortality (ICD-10 J41–J44)
- Hospital Admissions (any cause)
- ER Visits (any cause)

Methods

Data Management & Quality Control:

- Cleaned EHR data for 24,929 COPD patients
- Handled missing eFI data; conducted selection bias analysis
- Created analytic dataset of 18,278 patients

Selection Bias Check

Excluded patients were significantly older. Complete-case analysis adopted (no imputation).

Baseline Characteristics by Frailty Category

Mean ± SD or N (%)

Characteristic	Fit	Pre-Frail	Frail	P-value
N	3,554	8,768	5,956	
Age (years)	66.4 ± 6.8	71.2 ± 6.5	76.2 ± 6.1	<0.001
Female (%)	48.2	52.1	57.3	<0.001
CCI Score	2.1 ± 1.8	3.2 ± 2.1	4.8 ± 2.4	<0.001
eFI Score	0.07 ± 0.02	0.15 ± 0.03	0.28 ± 0.06	<0.001

Statistical Modeling:

- Implemented Firth's penalized logistic regression (`logistf`) for rare events
- Applied restricted cubic splines with 5 knots (`rms` package)
- Calculated AUC values and 95% CIs for outcomes
- R packages: `logistf`, `rms`, `dplyr`, `ggplot2`

Data Visualization:

- Created WFU-branded tables using `gt` and `ggplot2`
- Prepared presentation and poster for dissemination

Lessons Learned

About the Field:

- **Intellectual honesty** is essential — reporting modest results transparently builds credibility
- Predictive modeling involves trade-offs between **scalability and accuracy**
- Rare events require specialized methods (e.g., Firth's correction)

Personal & Professional Growth:

- Confidence in independent problem-solving with R
- Translating statistical jargon for clinical audiences
- Reproducible workflows: version control, documentation

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Github: https://github.com/xeriksen/predicting_outcomes_using_eFI